13-18 YEAR OLD PARENT IDI SUMMARY

Parents' Top Concerns for Their Youths *(excluding physical health concerns)*

1) **Social & Peer Influence**
   a) Worried about their child mixing with the wrong crowd, engaging in problematic behaviours such as smoking, vaping, or bullying
   b) Peer pressures such as lifestyle influences and financial status concerns
   c) Relationship with friends and fitting in socially
2) **Digital Device & Media Usage**
   a) Access to inappropriate content online
   b) Heavy device usage (on public transport, while walking)
3) **Mental/Emotional/Spiritual Health**
   a) Less willingness to share issues with parents
   b) Personal confidence and motivation
   c) Adherence to values (religion, family values)

Parenting in a Digital Age

1) **Whether technology helps or hinders parenting**
   Most parents saw technology as a "double-edged sword".
   a) **Helps**
      i) **Supports learning and skills development** through access to educational information and resources (through websites, ChatGPT etc.)
      ii) **Facilitates communication** with friends, family and teachers
      iii) **Promotes bonding:** Parents and youths can discuss what they see online
   b) **Hinders**
      i) **Exposure to inappropriate content** which may be outside parents' knowledge or control
      ii) **Risks of online peer pressure and cyberbullying**
      iii) **Excessive gadget use**, reducing meaningful offline interactions

2) **Technology's impact on 5 aspects of health**
   Overall, technology is viewed as having mixed impact on youths' health, where overuse is harmful. Parents placed high importance on maintaining a balance between youths' online and offline life through some of the following methods elaborated below.
   a) **Learn Well / Screen Time**
      i) Excessive screen time reduces physical activity
         - This in turn negatively impacts sleep, eating habits and learning outcomes
      ii) Some parents encourage outdoor activities (such as cycling or visits to the library) and enforce rules like no phone usage during meals
   b) **Eat Well**
      i) Technology allows access to recipes, nutritional information, calorie tracking apps
      ii) Possibility of body image disruption due to exposure to social media content
      iii) Gadget overuse may result in skipping meals
   c) **Exercise Well**
      i) Health apps and trackers (e.g. FitBit, HPB apps) promote exercise

ii) Excessive screen time leads to decreased outdoor activity

d) Sleep Well

i) Gaming or late-night screen use often lead to sleep deprivation

e) Oral Health

i) Generally minimal impact, but there is some positive impact from online research and product recommendations

3) Parental role in youth health and digital well-being

a) Importance of parents' role

i) Parents perceive that they are primary influencers of digital habits

ii) Early intervention (toddler/kindergarten age) is ideal; starting at 13-18 is late, but is still valuable nonetheless

iii) Parents feel they should guide teens by educating them on risks, setting screen-time limits, and encouraging offline activities

b) Youth independence and self-regulation

i) Parents noted that some children self-regulate (reduce screen time, sleep early, maintain healthy habits)

ii) However, parental monitoring remains necessary to ensure adherence

c) Challenges

i) Teenagers may resist rules; they may find loopholes in digital control apps

ii) Habits of youths are hard to break at an older age

d) Other stakeholders include peers, schools, other caregivers (grandparents, domestic helpers) and the government

4) Resonance with the Bond Well message

a) Overall, parents resonated with the Bond Well message

i) Parents agree that strong bonds provide emotional security and self-esteem

ii) Also that family helps cultivate children's good habits and values

b) Challenges in parent-youth bonding

iii) Youths tend to share less with parents, than when they were younger

iv) Negotiating rules becomes harder as children get older and need more flexibility

i) Conflicts often arise over digital device use

c) Ways to create a conducive home environment

v) Engage in shared activities and quality time

vi) Cultivate an environment of open communication and respect

5) Knowledge on guidelines for 5 aspects of health

a) Parents generally know the basic health guidelines:

i) Screen time of about 2 hours per day

ii) My Healthy Plate

iii) Physical activity of 150 minutes per week

iv) 8 hours of sleep per day

v) Brushing teeth twice daily, dentist visits twice a year or annually

b) Many parents are uncertain about the details of the guidelines or age-specific recommendations

c) Many admit that adherence is inconsistent

Testing Message House CTAs

1) Learn Well / Screen time

a) **Feedback on CTAs / Ideas**
   i) Many are already practising CTA 1 but effectiveness varies
   ii) Few parents proactively practise CTA 2

b) **Success factors**
   i) Clear communication and trust
   ii) Tapping into the youth's interests to encourage offline activities and quality time

c) **Barriers**
   i) Youths want privacy and autonomy; enforcing control may strain relationship with them
   ii) Parents' own overuse of screens undermines their role-modelling
   vii) Parents sometimes feel it is too late to intervene

d) **Youth receptivity**
   i) Most receptive to CTA 1: Parents being fully present is appreciated
   ii) Most resistant to CTA 3: Youths value independence and privacy and dislike parental interference

e) **Feedback on resources**
   i) Tools like parental control apps are helpful
   ii) Parents want more tips for handling resistance and starting conversations

2) Eat well / Nutrition

a) **Feedback on CTAs / Ideas**
   i) Most are practising the CTAs by preparing food at home, limiting snacks and moderating salt/oil
   ii) Many also moderate beverage intake, such as buying 0% sugar drinks

b) **Success factors**
   i) Starting healthy eating habits early
   ii) Involving youths in related activities, such as cooking and grocery shopping
   iii) Using role models such as sports idols to encourage healthy eating

c) **Barriers**
   i) Picky eating and individual preferences
   ii) Social influence, such as buying drinks with friends
   iii) Easy access to processed foods outside home

d) **Youth receptivity**
   i) Most resistant to CTA 2 due to preference for sweet drinks and comfort foods

e) **Feedback on resources**
   i) Resources like the Healthy 365 app are useful
   ii) Parents want stricter school policies like banning processed foods

iii) More practical ideas for healthier swaps and preparing food for picky eaters

3) Exercise Well / Physical Activity:

a) **Feedback on CTAs / Ideas**
   i) Most are practising CTA 1 and 3, citing activities like cycling and walking
   ii) CTA 4 is less practised due to family members' conflicting schedules
   iii) CTA 2 is less practised as youths may not want to join their parents' activities

b) **Success factors**
   i) Choosing activities aligned with the youth's interests
   ii) Incorporating gamified and creative activities, such as walking trails, Nintendo Switch, or YouTube workout videos

c) **Barriers**
   i) Schedule conflicts and lack of time
   ii) Limited access to public facilities
   iii) Youths' low interest in exercise or exercising with parents

d) **Youth receptivity**
   i) Most receptive to and excited by CTA 1 and 5
   ii) Most resistant to CTA 4 due to conflicting family schedules

e) **Feedback on resources**
   i) Parents are mostly aware of HPB initiatives and ActiveSG incentives, but usage is low due to time constraints or lack of interest
   ii) There is a lack of activities tailored for teens and parent-teen bonding

4) Sleep Well:

a) **Feedback on CTAs / Ideas**
   i) Most are practising CTA 3 as it is easy and can be consistently done
   ii) CTA 1 is commonly attempted as parents give reminders to sleep early, but with limited success
   iii) CTA 4 is most challenging as parents themselves often sleep late

b) **Success factors**
   i) Consistent reminders and check-ins
   ii) Early formation of healthy habits in phone usage
   iii) Creative strategies, such as planning an early brunch the next day so youths will sleep earlier

c) **Barriers**
   i) Academic demands and late-night studying
   ii) Teens set their own routines and ignore parent's reminders
   iii) Parents' own habits such as working late or watching TV before bed
   iv) Inconsistent sleep schedules on weekends

d) **Youth receptivity**
   i) Most resistant to CTA 1 as it involves device restrictions
   ii) Most receptive to CTA 3, especially novel sleep aids like reed diffusers or pillow sprays

e) **Feedback on resources**
   i) Parents find apps useful, such as Apple Health or Healthy 365

ii) School handouts are helpful but have limited impact
iii) More practical strategies such as information on foods that can help sleep, or access to sleep clinics

5) Oral Health:

a) **Feedback on CTAs / Ideas**
   i) Most are practising CTA 1 and 2
   ii) CTA 3 is more challenging
b) **Success factors**
   i) Early cultivation of oral hygiene habits from young
   ii) Teens may be self-motivated by appearance concerns or health consciousness
c) **Barriers**
   i) Private dental visits are expensive
   ii) Working parents may struggle to find time to bring children to dental visits
   iii) Youths have more autonomy and may ignore reminders about oral hygiene
d) **Youth receptivity**
   i) Most receptive to CTA 1
   ii) Some resistance to CTA 2 and 3 due to fear of dentists and reluctance to give up snacks and beverages
e) **Feedback on resources**
   i) Existing resources like Singapore Dentist website, HealthHub dental app, Polyclinic dental services are helpful
   ii) School dental services may not be sufficient
   iii) More financial support for private dental visits is desired

Communication Formats and Channels

1) **Parents strongly prefer digital communication / softcopy**
   a) Messaging apps (WhatsApp, Telegram) are top choices for easy sharing.
   b) Email is widely accepted
   c) Hardcopy materials (pamphlets) are considered obsolete
   d) Most are open to social media (Instagram Reels, Facebook posts, YouTube Shorts)
2) **Trusted Sources**
   a) Parents prefer health information from official sources (HPB, schools)
   b) Content should be engaging, not overly formal
3) **Other Influences**
   a) Parents follow doctors and influencers online but are aware that these sources may not always be accurate

Other Suggestions

1) Parents are **generally aware** of health-related resources and **appreciate** government efforts, but usage of existing resources is low
   a) To help with a lack of accessibility:
      i) Improve outreach/publicity
      ii) Add QR codes for easy access
      iii) Make content short and interesting

b) **To counter existing activities which do not appeal to teens:**
   i) Introduce more gamification
   ii) Offer activities for both parents and teens

2) **Parents express desire for more support in the following:**
   a) **Community platform**: Forums/Q&A for sharing experiences with other parents, success stories, statistics and norms
   b) **Practical help**:
      i) Tips for parent-child communication
      ii) Access to counsellors, psychologists, and interviews with parents
      iii) Support for special needs children